

Healthcare Claims Modernization Program — Program Closure Report

Portfolio simulation • fully populated program-management artifact

Closure Summary

The program completed its planned 16-month lifecycle and transitioned deliverables to accountable operational owners. Governance, financial controls, risk acceptance, knowledge transfer, support readiness, and benefit measurement responsibilities were formally closed or transferred.

Outcome Review

KPI	Plan / Baseline	Result	Outcome
Clean claim rate	83	94	+11 pts
Avg. adjudication days	6.8	3.9	-43%
Payment accuracy	97.2	99.3	99.3%
Manual touch rate	29	17	-41%
Critical defects at go-live	12	1	92% reduction
Provider inquiry volume	100	78	-22%

Benefits Realization

Benefit	Planned Annual Value	Forecast / Realized Value	Variance
Claims operations productivity	\$1,180,000	\$1,260,000	\$80,000
Reduced rework / adjustments	\$540,000	\$590,000	\$50,000
Platform maintenance reduction	\$410,000	\$425,000	\$15,000
Provider service efficiency	\$270,000	\$310,000	\$40,000

Lessons Learned

- Dependencies need named acceptance criteria, not only dates; this reduced ambiguity at handoffs.
- Capacity planning is most effective when specialist roles are modeled across all workstreams before commitments are made.
- Executive decisions close faster when the recommendation includes quantified impact and a decision deadline.
- Benefits ownership must be assigned during planning, not at closure, so data sources and baselines are available.
- Release readiness improves when operational support, communications, training, security, and rollback are evaluated in one gate.

Transition Actions

- BAU owners accepted operational procedures and support responsibilities.
- Open low-severity optimization items transferred to operational backlogs with owners and target dates.
- Benefits scorecards transferred to Finance and business benefit owners for 30/60/90-day validation.
- Program artifacts archived as the final source of truth; decision and risk histories retained for auditability.